

1. Administrative & Study Identification

Full Study Title: A Disease Registry of Patients With Mantle Cell Lymphoma **Short Title/Acronym:** SUMMIT **Protocol Number:** D8220R00004
NCT Number: NCT03816683
Sponsor Name: AstraZeneca
Investigational Product: Novel Therapies (including Bcl-2 inhibitors, BTK inhibitors, Immunomodulatory agents, and Phosphoinositide 3-kinase inhibitors)
Phase of Development: N/A (Observational / Patient Registry)
Medical Monitor: AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess treatment patterns, physician-reported clinical outcomes, and patient-reported health-related quality of life (HRQoL) among patients diagnosed with Mantle Cell Lymphoma (MCL) who are initiating novel therapy. **Secondary Objectives:** To evaluate event-free survival (EFS), overall response rate (ORR), complete response (CR) rate, treatment toxicity, and healthcare resource utilization when these agents are administered outside of a clinical trial.

2.2 Background & Rationale Newer targeted therapies (monotherapy or in combination) have been recently approved in the United States for the treatment of adult patients with mantle cell lymphoma (MCL) who have received at least 1 prior therapy. The approval of these newer therapies has a significant impact on treatment patterns, toxicity patterns, and overall survival in the MCL population. This registry helps generate real-world evidence to better understand the clinical impact and value of these therapies in routine clinical practice.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective Cohort Disease Registry) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 250 adult patients (227 enrolled) **Number of Sites:** ~50 academic (60%) and community (40%) settings in the United States **Estimated Study Start:** 01-Apr-2019 **Estimated Primary Completion:** 18-Mar-2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years old at the time of the first observed diagnosis of MCL. 2. Confirmed diagnosis of Mantle Cell Lymphoma (MCL). 3. A clinical decision has been made to initiate a novel therapy in the last 6 months (limited to Bcl-2 inhibitors, BTK inhibitors, immunomodulatory agents, or Phosphoinositide 3-kinase inhibitors) and treatment is ongoing at the time of enrollment.

4.2 Exclusion Criteria 1. Patients without a confirmed MCL diagnosis. 2. Patients treated exclusively with standard chemotherapy regimens without the inclusion of the specified novel agents. 3. Inability or unwillingness to provide informed consent or to use the patient portal for reporting.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Dosing and schedule as prescribed in real-world clinical practice by the treating physician for the specific novel agent. **Route of Administration:** Oral or Intravenous (dependent on the specific novel agent prescribed). **Duration of Treatment:** Maximum observation and follow-up of 24 to 60 months.

5.2 Concomitant & Prohibited Therapy Permitted: All standard-of-care concomitant medications are permitted, as this is an observational study of real-world practices. **Prohibited:** N/A (Treatment is at the discretion of the treating physician).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, Baseline Demographics, Clinical & Treatment History
Baseline	Day 0	Index Enrollment, Start of Registry Observation, Initial HRQoL Assessment
Interim	Quarterly	Efficacy Assessment (Physician-reported), Patient-Reported Outcomes (PRO-CTCAE), Healthcare Resource Use
End of Study	Month 24 to 60	Final Survival Status, Treatment Discontinuation, End of Observation

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Treatment patterns (rate of treatment interruptions in months and time on therapy) and HRQoL using the PRO version of the Common Terminology Criteria for Adverse Events (PRO-CTCAE) on a quarterly basis. **Measurement Method:** Aggregated patient data using descriptive statistics;

unique 5-digit code health states based on the PRO questionnaires.

7.2 Secondary & Exploratory Endpoints * Event-Free Survival

(EFS): Time from the start of novel MCL treatment to disease progression, death, or treatment discontinuation for any reason.

* **Overall Response Rate (ORR):** Frequency and proportion of patients with a complete or partial response based on physician assessment. * **Complete Response Rate (CR):** Frequency and proportion of patients achieving complete response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Patient perceptions of HRQoL and toxicity are collected via the PRO-CTCAE quarterly (specifically targeting muscle pain, joint pain, and heart palpitations with a 7-day recall). **Serious Adverse Events (SAEs):** Collected per standard pharmacovigilance practices for real-world observational registries. **Data Monitoring Committee (DMC):** Evaluated through standard sponsor internal review for real-world registries.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All enrolled and eligible patients initiating a novel therapy. **Sample Size Justification:** A sample of 250 patients was planned to provide adequate data for descriptive analyses of real-world outcomes across multiple academic and community centers. **Handling of Missing Data:** All survival and treatment interruption outcomes are descriptive. Missing data in patient-reported questionnaires are handled through standard practices for observational research (e.g., analyzed based on available data).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Pharmacoepidemiology Practices (GPP), Good Clinical Practice (GCP), and the Declaration of Helsinki. **Monitoring Plan:** Observational monitoring via an electronic data capture system; continuous patient outcome collection through a dedicated novel patient portal. **Audit Trail:** Standard electronic data capture audit trails and regular data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: D8220R00004 **Registry Number:** NCT03816683 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** SUMMIT: A Disease Registry

12. Clinical Framework (Mantle Cell Lymphoma Specific)

Primary Condition: Mantle Cell Lymphoma (MCL) **Diagnosis Threshold:** Confirmed diagnosis of MCL via standard pathological criteria **Medical Methodology:** Administration of targeted novel therapies (Bcl-2, BTK, PI3K inhibitors, or immunomodulators) **Optimization Target:** Disease control, increased event-free survival (EFS), and improved health-related quality of life (HRQoL)

13. Study Procedures & Methodology

Management Pathway: Standard clinical treatment pathways for MCL in academic and community real-world settings **Education Protocol (HCP):** N/A (Observational Real-World Evidence Registry) **Education Protocol (Patient):** Self-learning and portal use training for PRO-CTCAE reporting **Quality Audit Mechanism:** Data collection via a novel patient portal assessing symptoms, treatment toxicity, and healthcare resource use

14. Observation & Follow-Up Schedule

Total Duration: 24 to 60 months of follow-up **Onsite Visit Cadence:** Aligned with standard of care real-world clinical visits **Remote Monitoring Frequency:** Ongoing via patient portal **Checklist Review Interval:** Every 3 months (Quarterly PRO-CTCAE questionnaire review)

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				